

Anonymized Case Description by Reconfig

Medical Affairs Operating Model Redesign
in a Global Pharmaceutical Company

*Summary Document for Anonymized Survey Dataset
Prepared for Research Publication*

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1. Organization Profile

The case organization is the domestic Medical Affairs office of a large, global biopharmaceutical company headquartered in Asia with operations spanning more than 70 countries. The specific unit under study is the company's home-country Medical Office, responsible for medical-scientific engagement, evidence generation, post-marketing surveillance, and stakeholder communication across multiple therapeutic areas including oncology, rare diseases, gastroenterology, neuroscience, and vaccines. The unit employs approximately 142 staff and 17 external consultants organized across six major functional areas.

The Reconfig research ODT for this case is hosted at: <https://app.reconfig.no/design/org/1599>

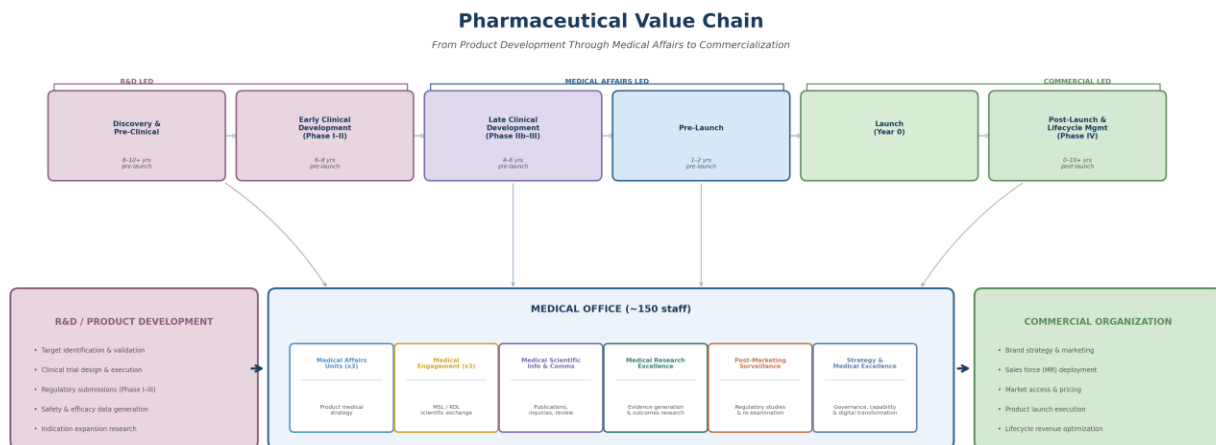


Figure 1. Pharmaceutical Value Chain — From Product Development Through Medical Affairs to Commercialization

2. Context and Motivation for the Study

The Medical Office underwent a major organizational restructuring effective April 2025, driven primarily by financial optimization pressures. The restructuring coincided with a broader corporate transformation that created parallel medical organizations at the global, international, and domestic levels, generating significant structural complexity and coordination risk. Approximately 70% of the domestic medical budget was shifting to a newly created global medical function, creating tensions around local autonomy, accountability, and decision-making authority.

The restructuring was particularly consequential because the home country has unique regulatory requirements, specifically a mandatory post-marketing drug re-examination process (7-8 years post-approval) where failure can result in market withdrawal. This creates a critical accountability vs

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authority mismatch: global headquarters sets standards and controls budgets, while the local office retains execution responsibility and regulatory liability.

3. Organizational Structure Under Study

The new operating model consolidated the Medical Office into six integrated functional areas:

- 1. Medical Affairs Units (3 therapeutic-area units):** Own the medical strategy for products, translate global strategy into local medical plans, and determine medical activities per product. Classified as revenue centers.
- 2. Medical Engagement (3 field-based units):** Execute scientific exchange with healthcare professionals (HCPs) and key opinion leaders (KOLs) through Medical Science Liaisons (MSLs) and nurse educators, bringing field insights back into the organization. Classified as a profit center.
- 3. Medical Scientific Information & Communications:** Responsible for medical content management, scientific publications, congress presentations, pharmaceutical material review, and responding to inquiries from healthcare professionals. Contains three sub-units: scientific information and communications, medical affairs response (call center and inquiries), and pharmaceutical material review.
- 4. Medical Research Excellence:** Designs and executes integrated evidence generation strategies including investigator-initiated research, company-sponsored clinical studies, database studies, and outcomes research. Contains two evidence generation sub-units.
- 5. Post-Marketing Surveillance Strategic Management:** Conducts mandatory post-marketing surveillance studies, manages regulatory compliance, monitors study sites, and handles re-examination applications. Contains three study management teams plus compliance and operational excellence sub-units.
- 6. Strategy & Medical Excellence:** Leads organizational strategy, planning, budgeting, governance, compliance, capability development, digital transformation, and new product value planning. Contains sub-units for strategy coordination, medical excellence and innovation, medical governance, and new product value planning.

Each unit was explicitly classified by strategic value type (investment center, revenue center, profit center, service center, or support center), and all functions were categorized as either strategic or support functions.

4. Dimensions of Interest for the Research

The case is rich across several dimensions central to operating model optimization:

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4.1 Work Process Performance and Operating Model Design

The organization defined approximately 70+ discrete work activities spanning all six functional areas, each mapped to specific sub-units and roles. The survey instrument was designed to capture how time and effort are actually distributed across these activities versus intended allocation. The workshop agenda explicitly focused on comparing “current vs. required work-effort distribution” and identifying differences between intended and actual focus areas.

4.2 Structural Coordination and Collaboration Patterns

The study employed Organizational Network Analysis (ONA) to map actual collaboration and dependency patterns across the 150-person organization. The survey was explicitly designed to “map how members collaborate with each other” following the 2025 reorganization. Key coordination dimensions include:

- Cross-functional interfaces (with R&D, commercial, regulatory, and external stakeholders)
- Bottlenecks, overloads, and informal hubs
- Collaboration patterns that should be “protected, not broken by default” even as reporting lines change
- Tension points between the domestic office, the global function, and regional business units

4.3 Work Consolidation and the Human vs AI Balance

A central design question was the optimal balance between human intelligence (HI) and AI-enabled support across medical affairs work. The workshop explicitly explored:

- Scientific knowledge synthesis and signal detection
- Insight capture, structuring, and reuse
- Preparation and follow-up of scientific interactions
- Identifying work that is primarily judgment-based (human-led) versus repeatable or analytical (potentially AI-supported)
- Scalability without proportional headcount increases

4.4 Learning and Capability Development

The strategy includes a dedicated focus on talent and capability development:

- Building a leadership and talent pipeline through structured development programs
- Cross-functional knowledge sharing frameworks between Medical Affairs, R&D, and Commercial
- Product and disease-area education for internal teams
- Digital capability development and training

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- Evolution of the MSL role from near-launch support to early scientific engagement (up to 5 years pre-launch)

4.5 Operating Model Scenarios and Lifecycle-Based Organization

The workshop explored alternative ways of organizing Medical Affairs work:

- Lifecycle-based vs. asset-focused organization of work, priorities, and decision-making
- Early-asset MSL engagement models (3-5 years pre-launch)
- Degree of centralization versus embedding of MSL capabilities within therapeutic-area units
- The shift from Medical Affairs as near-launch support to Medical Affairs as a strategic pillar alongside R&D and Commercial

4.6 Accountability vs Authority Alignment Under Matrix Structures

The case provides a natural experiment in how global-local matrix reporting affects operating model performance. The domestic office faces a structural tension where:

- Standards, frameworks, and budgets are controlled globally
- Local execution, regulatory interface, and de facto accountability remain domestic
- This creates coordination risks particularly acute for early-lifecycle products and regulatory-unique local requirements

5. Survey and Data Collection

A collaboration survey was administered in Q1 2026, approximately one year after the new organizational structure was implemented. The survey was:

- Administered to all 142 staff of the Medical Office (But none of the 17 consultants)
- Translated into the local language across multiple review iterations
- Designed to capture collaboration patterns, work-effort distribution, and organizational network data
- Positioned explicitly as non-evaluative (“not meant for personal evaluation”)
- Achieved a 98% response rate among survey respondents
- Analyzed using the Reconfig Organizational Digital Twin (ODT) platform that enabled scenario modeling and visualization

The project delivery workshop convened 8 senior leaders representing the full 159-person organization to validate current-state findings, compare operating model scenarios, and develop a transformation roadmap.

6. Industry Significance

This case represents a broader trend in the global pharmaceutical industry where Medical Affairs functions are evolving from reactive, near-launch support roles toward strategic pillars that steward scientific data, knowledge, and insights across the full product lifecycle. The case captures this transition at a moment of high organizational complexity, simultaneous corporate restructuring, global-local tension, AI/digital transformation, and regulatory uniqueness, making it particularly valuable for studying how operating model design choices affect work process performance, coordination, learning, and organizational effectiveness.